



Guidelines

Adult Diabetic Ketoacidosis (DKA)

Reference #: HRM025

Scope

Site	Service/Department/Unit	Disciplines
SCGH	Hospital Wide	Medical, Nursing, Pharmacy
Out of Scope	This guideline must NOT be used in: - Children (<16 years) even if they have DKA. Seek specialist advice. - In the management of Hyperglycaemic hyperosmolar state (HHS) here.	

Introduction

This guideline provides guidance to all Sir Charles Gairdner Osborne Park Hospital Care Group (SCGOPHCG) staff involved in the management of DKA. This document supports and complies with related legislation, policy and other related documents [here](#).

1.0 General Information

Continuous cardiac monitoring must be instituted when [intravenous potassium chloride](#) is administered at a rate greater than 10 mmol/hour.

OPH patients diagnosed with DKA/HHS require early liaison with SCGH Endocrinology. Routine continuous cardiac monitoring for DKA / HHS patients is not available at OPH per the [SCGOPHCG High Risk Medicines Policy](#). Patients requiring intravenous potassium administered at a rate greater than 10 mmol/hour must be transferred to SCGH.

Diabetic Ketoacidosis (DKA) is a complex disorder, commonly seen in patients with Type 1 diabetes (although patients with type 2 diabetes and rarely Gestational Diabetes may also be affected) and can be life threatening if not treated promptly and properly.

Primary abnormalities are profound insulin deficiency, reduced uptake of glucose by muscle cells, accelerated glucose production from glycogen by liver cells and increased conversion of free fatty acids into ketone bodies in the liver. These processes manifest as extreme hyperglycaemia, ketonaemia, ketonuria, acidosis, dehydration, and electrolyte imbalance.

2.0 Indications

- This guideline is for the treatment and management of adults with DKA.
 - Seek advice from senior clinician or diabetes specialty unit in cases of uncertainty.
- This guideline can be used in pregnancy. Refer to specific section below.

3.0 Guideline information

Staff must adhere to the following guideline when managing an adult patient with Diabetic Ketoacidosis (DKA). The guideline must be adapted to the clinical situation where necessary.

All management must be recorded in Adult Diabetic Ketoacidosis (DKA) Guidelines and Management Record (MR849), see [Appendix 1](#). These documents are designed to allow both prescriptions and relevant observations to be recorded together.

The supplementation of potassium may be via the enteral (e.g. oral, nasogastric, PEG) or intravenous route. Intravenous replacement of potassium should only be prescribed when there is no suitable alternative route following appropriate clinical assessment by a medical officer.

Refer to the "SCGH Patient Flow/Disposition Plan" in [Appendix 2](#) for management of mild/moderate and Severe DKA patients at SCGH.

4.0 Diagnosis (Entry) Criteria

- Blood glucose: > 11 mmol/L **or** known diabetes, **and**
 - *Venous bicarbonate (HCO_3): < 18mmol/L **and/or** venous pH < 7.35*, **and**
 - Blood (capillary) ketones: ≥ 3 mmol/L or urine ketones $\geq 2+$.
- *Note: The standard of care is the analysis of venous bicarbonate. Arterial blood gases should ONLY be considered in patients with impaired consciousness.

5.0 Resolution (Exit) Criteria

- Resolution of capillary ketonaemia (< 0.6 mmol/L). This should occur within 24 hours, **and**
- Correction of acidosis [venous bicarbonate (HCO_3) >18 mmol/L and/or venous pH > 7.35], **and**
- Patient is eating and drinking normally, **and**
- Treatment/resolution of underlying or precipitating cause (where appropriate), **and**
- Discharge planning including planned diabetes review post-discharge.

6.0 New Principles for the Management of DKA

- Aim to correct the cause of the acidosis, i.e. the ketonaemia.
- Insulin is given as a standard dose per kg until ketones are cleared (< 0.6mmol/L).
- **Aim to cease high-dose fixed rate insulin infusion as soon as ketones are cleared and convert to subcutaneous insulin ([click here for guide](#)).**
- **If intravenous insulin is still required (e.g. patient is not yet eating or drinking normally), medical staff must convert the patient to a variable rate intravenous insulin infusion with glucose 10% as per separate guideline and chart i.e. convert to [Adult Variable Rate Insulin guideline](#) & management record.**
- **Seek advice from Endocrinology or a senior clinician regarding obstetric intravenous insulin. Click [here](#) for DKA management in Pregnancy.**
- Continue usual subcutaneous basal insulin in addition to IV insulin or start basal insulin at outset if patient is newly diagnosed.
- Monitor bedside capillary ketones and glucose with appropriate equipment / meters.
- Use sodium chloride 0.9% for resuscitation, not colloid.
- Give intravenous sodium chloride 0.9% and glucose 10% together if ketones are still present (>0.6 mmol/L) and blood glucose level is <14 mmol/L.
- Where available, ensure the patient receives early diabetes specialist team review and appropriate post-discharge follow-up.

7.0 Essential Initial Assessment to Determine the Severity of DKA

The following assessment must be carried out to determine the severity of the patient's DKA:

- Blood (capillary) ketones
- Blood glucose level (BGL)
- Venous bicarbonate (HCO_3)*
- Venous pH*
- Potassium - beware of initial low potassium. If level is $< 3.5\text{mmol/L}$, call for senior medical advice immediately
- Serum osmolality
- Creatinine
- Systolic blood pressure (SBP)
- Glasgow Coma Scale (GCS)

*Note: The standard of care is the analysis of venous bicarbonate and pH. Arterial blood gases should only be considered in patients with impaired consciousness.

Table 1: Severity Assessment of DKA*

Parameter	Severity	
	Mild to Moderate	Severe [^]
Systolic blood pressure (SBP)	$> 90\text{ mmHg}$	$< 90\text{ mmHg}$
Venous pH	$7.1 - 7.35$	< 7.1
Blood (capillary) ketones	$3 - 6\text{ mmol/L}$	$> 6\text{ mmol/L}$
Venous Bicarbonate (HCO_3)	$10 - 18\text{ mmol/L}$	$< 10\text{ mmol/L}$
Glasgow Coma Scale (GCS)	15	< 15
Serum Potassium (K^+)	$> 3.5\text{ mmol/L}$ on admission	$< 3.5\text{ mmol/L}$ on admission

***To be read in conjunction with the SCGH DKA Flow Pathway/disposition plan [in Appendix 2](#).**

[^]If any of the SEVERE criteria are present, early senior medical officer advice must be sought for consideration of admission to critical care area i.e. ICU or General HDU.

8.0 Initial Management (1st Hour – Fluids / Potassium / Insulin)

1. Establish 2 large bore intravenous cannulas. Insulin infusions and glucose infusions must run on separate pumps. 1 cannula will require a 3-way cannula extension (Chooks foot).
2. Administer 500 mL to 1000 mL of sodium chloride 0.9% over 15 minutes if systolic BP is < 90 mmHg.
3. If systolic BP remains < 90 mmHg, repeat step 2 and call for senior medical advice. Septic shock and heart failure can be potential causes and ICU involvement may be indicated.
4. If systolic BP is >90 mmHg, **prescribe sodium chloride 0.9% (with or without potassium supplementation) according to [Table 2](#)**. Adjust the rate of fluid resuscitation according to the patient's age, fitness, co-morbidities, pregnancy status and volume status.
5. Consider the use of urinary catheter if no urine is passed for 2 hours or incontinent.
6. Consider nasogastric tube insertion and the potential for aspiration if conscious state is impaired.
7. Consider thromboprophylaxis in the elderly or "high risk" patient.
8. Look for and treat likely precipitants of DKA. Screen for infection and silent ischaemia. Note: White cell count is often elevated in DKA and does not necessarily confirm infection.
9. Consider critical care (GHDU/ICU) involvement if:
 - In severe DKA (see [Table 1](#)).
 - Young (16 - 25 years old).
 - Pregnant.
 - GCS <12.
 - In shock.

9.0 Intravenous Fluid and Potassium Management

Potassium levels will drop rapidly with fluid and insulin replacement; consequently, it is important to replace potassium early as low potassium can cause serious harm or death. Initial potassium is often normal or high, but the patient's total body potassium is low.

Note: The maximum rate of potassium chloride administration via peripheral intravenous lines is 10 mmol per hour. One exception is the management of DKA where up to 20 mmol per hour of potassium chloride can be administered peripherally.

Consider central line access for patients in Critical Care Areas (ED, GHDU, ICU) likely to require prolonged high dose (>20 mmol per hour) intravenous potassium chloride.

Continuous cardiac monitoring must be in place during administration of > 10 mmol/hr potassium chloride or if serum potassium is > 5.5 mmol/L or < 3 mmol/L).

- Commence intravenous glucose 10% infusion at 125 mL/hr if blood glucose level (BGL) < 14 mmol/L.
- Increase intravenous glucose 10% infusion rate to 187 mL/hr if BGL < 5 mmol/L.
- Reduce sodium chloride 0.9% proportionately.
- Consider reducing the rate of fluid replacement in the following patient groups; adults aged 16 to 25 years, elderly, pregnant, or those with serious co-morbidities (e.g. cardiac or renal disease).

Table 2: Intravenous fluid and potassium infusion

Bag	Fluid	Serum potassium level and potassium chloride supplementation required			Sodium chloride 0.9% (+/- potassium chloride) infusion rate	
		Greater than (>) 5.5 mmol/L	3.5 to 5.5 mmol/L	Less than (<) 3.5 mmol/L <i>**Senior Medical review required</i>	Rate WITHOUT concurrent glucose 10% (mL/hr)	Rate WITH concurrent glucose 10% (mL/hr)
1 st	1 L sodium chloride 0.9%	Nil	Nil	Nil	1000	875
2 nd **	1 L sodium chloride 0.9%	Nil	40 mmol	40 mmol	500	375
Cardiac monitoring must be instituted when intravenous potassium chloride is administered at a rate of greater than 10 mmol per hour.						
3 rd **	1 L sodium chloride 0.9%	Nil	40 mmol	40 mmol	500	375
4 th	1 L sodium chloride 0.9%	Nil	40 mmol	40 mmol	250	125
5 th	1 L sodium chloride 0.9%	Nil	40 mmol	40 mmol	250	125
6 th	1 L sodium chloride 0.9%	Nil	40 mmol	40 mmol	167	125
**Rate of potassium infusion through peripheral lines must be no more than 20 mmol potassium chloride per hour in DKA. The patient must be on a cardiac monitor if receiving > 10 mmol/hr potassium chloride, or if serum potassium is > 5.5 mmol/L or < 3 mmol/L.						

10.0 INSULIN MANAGEMENT

1. Add 50 units of Actrapid® to 50 mL of sodium chloride 0.9%.
2. **Commence fixed rate intravenous insulin at 0.1 units/kg/hour.**
3. Prescribe subcutaneous basal (long acting) insulin on day 1, on the Subcutaneous Insulin Order and Blood Glucose Monitoring Chart. Continue patient's usual dose OR commence [for newly diagnosed patient per subcutaneous conversion guide here](#).
4. Disconnect ALL continuous subcutaneous insulin infusion (CSII) pumps. DO NOT attempt to use the pump without specialist diabetes team input.

Important: Continue fixed rate insulin until ketones are cleared (<0.6 mmol/L). Then, if the patient is eating and drinking normally, convert to subcutaneous insulin ([see Conversion to](#)

[subcutaneous insulin](#)). If the patient is not yet eating and drinking normally, convert to [Adult Variable Intravenous Insulin](#) per the guideline and management record with glucose 10% to avoid hypoglycaemia.

11.0 Ongoing Management Considerations

1. Measure capillary ketones and glucose hourly.
2. Repeat venous blood gases (VBG) 1 hour after initial treatment and then 2 hourly until DKA is resolved. If DKA is severe, continue monitoring VBG hourly.
3. Aim to reduce capillary ketones by 0.5 mmol/hour.
 - If ketones **are** falling ≥ 0.5 mmol/L/hour:
 1. Continue with current insulin infusion rate.
 2. Alert medical officer once ketones are ≤ 0.6 mmol/L.
 3. Stop fixed-rate DKA insulin infusion once ketones < 0.6 mmol/L.
 4. When fixed rate infusion ceased, follow insulin advice per point [14.0](#).
 - If ketones **are NOT** falling by 0.5 mmol/L/hour:
 1. Check insulin pump is working/connected and check all infusion lines.
 2. Check residual insulin volume is correct.
 3. Seek senior medical review to consider adjusting fixed insulin rate.
 - increase insulin by 1 unit/hr (between hours 1 to 12) or 0.5 unit/hr (between hours 12 to 24).
4. Seek senior medical advice early.
5. In most cases bicarbonate administration is NOT helpful and is potentially dangerous. This should only be used in critical care areas i.e. ICU/GHCU/ED after discussion with the consultant in charge of the patient's care.

12.0 Serious Complications of DKA

- **Potassium** – both hyperkalaemia and hypokalaemia are potentially life-threatening. The patient's potassium level must be monitored carefully. It is expected that potassium may decline with treatment. Seek advice from senior medical staff early when it occurs.
- **Hypoglycaemia** – Blood glucose can fall rapidly with DKA treatment. When this occurs, use additional glucose **10%** when BGL < 14 mmol/L rather than reducing rate of insulin infusion.
- **Cerebral oedema** – especially in younger patients, it is rare but has a high mortality rate. Seek specialist advice early.
- **Pulmonary oedema** – is uncommon but cautious fluid resuscitation is necessary if the patient also has cardiac or renal impairment.

13.0 Treatment of Underlying Precipitants

- Elevated WCC (white cell count), fever and abdominal pain commonly occur in DKA and don't necessarily indicate the presence of infection or intra-abdominal pathology.
- Careful examination and screening for underlying causes and regular monitoring and re-evaluation is necessary.
- Consider underlying causes such as sepsis (including lower limb cellulitis and meningitis); silent ischaemia; and toxins (alcohol).
- If infection is present, refer to the [Therapeutic Guidelines](#) or [Statewide Medicines](#)

[Formulary.](#)

14.0 Conversion to Subcutaneous Insulin

Once ketones have been cleared, convert patient from this DKA guideline to an alternative treatment plan:

1. If the patient is **NOT** eating and drinking normally, medical staff must convert the patient to a **variable rate** intravenous insulin infusion with glucose 10% as per separate guideline [click here](#) & chart.

Note: The infusion rate will need to be manually changed on the variable rate IV insulin chart to the current equivalent dose administered as part of the DKA guideline.
2. Once the patient is well hydrated and resumed eating normally, convert to subcutaneous insulin guidelines as follows:
 - a. **For patients with known diabetes on multiple daily injections:**
 - o Recommence usual subcut **bolus** insulin with the patient's next meal.
 - o Cease IV insulin infusion 30 minutes after bolus injection.
 - o Note: If no basal insulin has been given for >24 hours, aim to convert to subcut insulin with evening meal and give usual basal dose, **OR** convert at breakfast with half of the usual basal dose and then give remainder of the dose with the patient's evening meal.
 - b. **For patients who are on CSII (insulin pump) therapy:**
 - o Consider pump failure as a cause of DKA; evaluate this before restarting the pump. Seek advice from the Specialist Diabetes Team/Endocrinology.
 - o If appropriate, recommence pump when ketones are cleared and the patient is eating and drinking.
 - o Cease IV insulin infusion 30 minutes after recommencing pump.
 - c. **For patients with newly diagnosed type 1 diabetes:**
 - o Commence **basal** insulin (e.g.: Optisulin® or Levemir® 10 units once-a-day) on the day of admission.
 - o Commence bolus insulin with the next meal.
 - o Cease IV insulin infusion 30 minutes after bolus injection.
 - o Suggested starting doses:
 - Bolus (meal-time) insulin: 4 to 6 units of rapid-acting insulin analogue.
 - Basal (once a day) insulin: 10 units Optisulin® or Levemir®.
 - o NB: Levemir® or Protaphane® preferred in pregnancy

15.0 DKA in Pregnant Woman

Pregnancy is a state of insulin resistance, accelerated starvation and respiratory alkalosis, especially in the late second and throughout the third trimester. Several hormones that are increased in pregnancy drive this e.g. human placental lactogen, prolactin, growth hormone and cortisol.

- Pregnant women with diabetes are more prone to severe and rapidly progressive episodes of DKA at lower glycaemic levels and can even develop DKA with "normal" BGL levels (i.e. euglycaemic DKA).

- Fetal mortality from maternal DKA is as high as 30% and complications include fetal hypoxia, acidosis, preterm delivery and neonatal intensive care unit admission.

In addition to usual care described above:

1. At presentation, the SCGH Endocrinology on call physician and KEMH on call physician/obstetric team should be consulted and the SCGH surgical team made aware if delivery imminent. Refer to [KEMH DKA management here](#).
2. Pregnant women who meet criteria for severe DKA or have sepsis should be managed in a critical care area i.e. ICU/GHCU.
3. Continuous foetal monitoring should be considered. Cardiotocography Monitoring (CTG) requires an accredited / approved midwives, obstetric registrar or consultant obstetrician to review, interpret and sign off results - not available at SCGH. Discuss fetal monitoring and management requirements with KEMH obstetrics.
4. The timing of delivery needs to be individualized based on multiple factors including gestational age, maternal condition (whether the mother is responding to aggressive therapy or deteriorating) and fetal condition (whether the fetal heart rate pattern is improving or deteriorating).
5. Glucocorticoids and betamimetics should be avoided during DKA as they will worsen hyperglycaemia. If women remain at high risk of preterm delivery glucocorticoids for foetal lung maturation should be considered after resolution of the maternal metabolic derangement.

Whilst evidence is limited in DKA in pregnancy it should be noted that pregnancy induces a respiratory alkalosis and a pH of <7 represents severe acidosis and bicarbonate may be considered in an ICU setting.

Related Documents

Legislation:

- [Medicines and Poisons Act 2014](#)
- [Medicines and Poisons Regulations 2016](#)
- [Health Services Act 2016](#)

Authorising Policy and Standard/s:

- [Mandatory Policy MP0077/18 Statewide Medicines Formulary Policy](#)
- [SCGOPHCG Formulary Management Procedures MMP013](#)
- [SCGOPHCG High Risk Medication Policy](#)
- [SCGOPHCG Medicines Management Policy and Procedure](#)
- NSQHS Standards 1, 4

Standards, procedures, guidelines:

- [SCGOPHCG Nursing Practice Guideline No. 51 Medication Management](#)
- [SCGOPHCG Nursing Practice Guideline No. 4 Intravenous Therapy](#)
- [SCGOPHCG Nursing Practice Guideline No. 2 Diabetes Patient Management](#)
- Adult Diabetic Ketoacidosis (DKA) Guidelines and Management Record (MR 849) SCGH
- [Adult Variable Rate Intravenous Insulin Guideline](#) and Management Record SCGOPHCG
- [Adult Hyperosmolar Hyperglycaemic State Guideline](#) and Management Record (MR 848) SCGH

- [SCGOPHCG SGLT2 inhibitors \(gliflozins\) and euglycaemic ketoacidosis guideline](#)
- [Diabetes in Pregnancy. KEMH Clinical Practice Guideline](#)
- [PCH Diabetic Ketoacidosis Protocol](#)

References

1. Garrsion, E et al (2014). Inpatient management of women with gestational and pregestational diabetes in pregnancy- review and expert opinion. Curr Diab Rep 14:457.
2. The Management of Diabetic Ketoacidosis in Adults. London, Joint British Diabetes Societies Inpatient Care Group. 2013. Retrieved from: <https://www.diabetes.org.uk/resources-s3/2017-09/Management-of-DKA-241013.pdf>
3. The King Edward Memorial Hospital Diabetes in Pregnancy Clinical Practice Guideline. Diabetic ketoacidosis (DKA) management pg 23 – 29. 2018 <https://healthpoint.hdwa.health.wa.gov.au/policies/Policies/NMAHS/WNHS/WNHS.OG.Diabetes.pdf>
4. The Royal Women's Hospital - Melbourne DKA in pregnancy clinical guideline.

Authorisation & Version Control

Policy Sponsor	Director of Clinical Services				
Policy Contact	Drugs and Therapeutics Committee Executive Officer (Senior Pharmacist)				
Date First Issued:	09/09/2019	Last Reviewed:	15/08/2023	Review Date:	15/08/2026
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Standards Applicable:	NSQHS Standards (Version 2): 				

For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

This document can be made available in alternative formats on request for a person with a disability.

Appendix 1. Adult DKA Management Record

DO NOT WRITE IN MARGIN

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M583
05/21

SIR CHARLES GAIRDNER OSBORNE PARK HEALTH CARE GROUP		USE PATIENT LABEL WHEN AVAILABLE	
ADULT DIABETIC KETOACIDOSIS (DKA) GUIDELINES AND MANAGEMENT RECORD		URN:	
		Family name:	
WARD:		Given name(s):	
CONSULTANT:		Gender:	DOB:
Prescriber to print patient name and check label correct.			
These guidelines are for the treatment of adults with diabetic ketoacidosis (DKA) and are designed to allow both prescriptions and relevant observations to be recorded together. The guideline should be adapted to the clinical situation where necessary (refer to related guideline - HRM025). Seek advice from senior clinician/diabetes specialty units in cases of uncertainty. This guideline can be used in pregnancy. Refer to specific section <u>Continuous cardiac monitoring must be instituted when intravenous potassium chloride is administered at a rate greater than 10mmol/hour.</u> To be read in conjunction with the "SCGH Patient Flow / Disposition Plan" in Appendix 2 of guideline HRM025. OPH patients diagnosed with DKA / HHS require early liaison with SCGH Endocrinology. Routine continuous cardiac monitoring for DKA / HHS patients is not available at OPH per the <u>OPH High Risk Medicines Policy</u>. Patients requiring intravenous potassium administered at a rate greater than 10mmol/hour must be transferred to SCGH. This guideline must not be used: - in children (<16years) even if they have DKA, seek specialist advice. - In the management of Hyperglycaemic hyperosmolar state (HHS) see MR848.			
1. DIAGNOSIS (ENTRY) CRITERIA: Tick boxes if criteria present, re-consider diagnosis if <3 criteria - Blood glucose: > 11mmol/L OR known diabetes, and ☐ - Venous bicarbonate: (HCO₃) < 18mmol/L and / or venous pH < 7.35*, and ☐ - Blood (capillary) ketones: ≥ 3mmol/L or urine ketones ≥ 2+ ☐ *Note: The standard of care is the analysis of venous bicarbonate. Arterial blood gases should ONLY be considered in patients with impaired consciousness			
2. RESOLUTION (EXIT) CRITERIA - Resolution of capillary ketonaemia (<0.6mmol/L). This should occur within 24 hours, and - Correction of acidosis (venous bicarbonate (HCO₃) >18mmol/L) and/or venous pH > 7.35, and - Patient eating and drinking normally, and - Treatment / resolution of underlying or precipitating cause (where appropriate), and - Discharge planning including planned diabetes review post-discharge.			
3. NEW PRINCIPLES OF MANAGEMENT OF DKA - Aim is to correct the cause of the acidosis i.e. the ketonaemia. - Insulin is given as a standard dose per kg until ketones are cleared (< 0.6mmol/L). - Aim to cease high-dose fixed rate insulin infusion as soon as ketones are cleared and convert to subcutaneous insulin (see box 11 on page 4 for guide). - If intravenous insulin is still required (e.g. patient is not yet eating or drinking normally), medical staff must convert the patient to a variable rate intravenous insulin infusion with glucose 10% as per separate guideline & chart i.e. convert to <i>Adult Variable Rate Intravenous Insulin guideline and management record</i>. - Seek advice from Endocrinology or a senior clinician regarding obstetric intravenous insulin (see guideline HRM025 for more information). - Continue usual subcutaneous basal insulin in addition to IV insulin or start basal insulin at outset if newly diagnosed. - Monitor bedside capillary ketones and glucose with appropriate equipment/meters. - Use sodium chloride 0.9% for resuscitation, not colloid. - Give intravenous sodium chloride 0.9% and glucose 10% together if ketones are still present (>0.6mmol/L) and blood glucose level is <14mmol/L. - Where available, ensure the patient receives early diabetes specialist team review and appropriate post-discharge follow-up.			

ADULT DIABETIC KETOACIDOSIS (DKA)
GUIDELINES AND MANAGEMENT RECORD

MR849

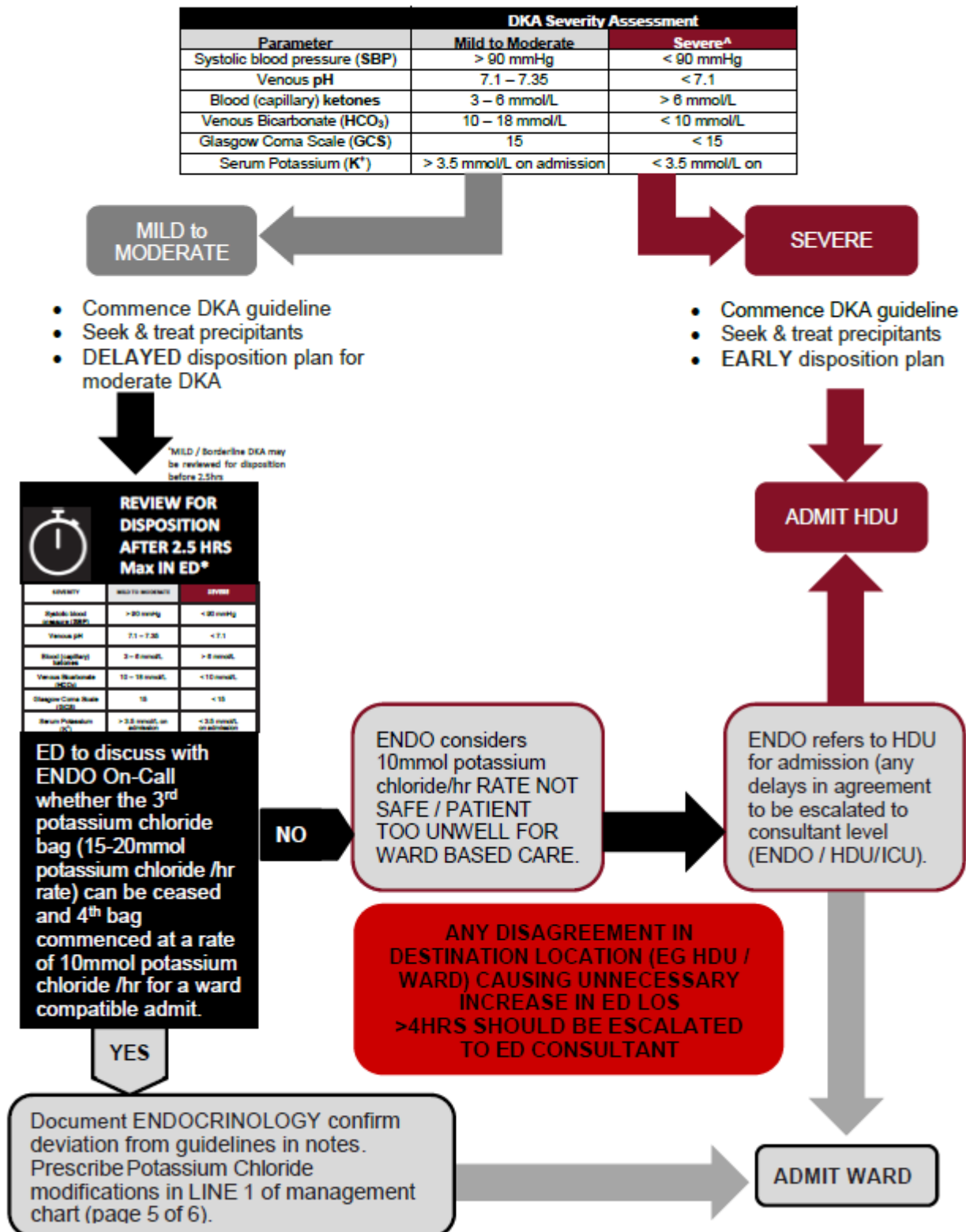
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HIMS approved March 2021

Appendix 2. Adult DKA Flow Pathway/Disposition Plan



SCGH Diabetic Ketoacidosis (DKA) FLOW PATHWAY / DISPOSITION PLAN TO BE USED IN CONJUNCTION WITH ADULT DKA GUIDELINE MR849



v1.0 This plan has been agreed by HOD ED / Endocrine / ICU / HDU / Exec